

Peri-procedure and Temporary Medication-Hold Policy

Authorization requirements for procedures, acute illness, interruption, and restart

Document ID	Version	Effective	Review due
NB-MED-204	1.0	2026-08-17	2027-08-17

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Owner: Peri-procedure Cardiology Lead
Approved by: Fictional Clinical Governance Committee
Classification: Training / synthetic corpus

Document control and retrieval metadata

Field	Value
Canonical title	Peri-procedure and Temporary Medication-Hold Policy
Document ID	NB-MED-204
Keywords	procedure, surgery, dental, anticoagulant, hold, restart, fasting, acute illness
Authority	Peri-procedure Cardiology Lead
Applicability	Training simulations only
Relationship	Standalone extension; existing corpus documents remain unchanged

Retrieval rule

Retrieve whenever a procedure, fasting instruction, acute illness, medication interruption, or restart question is present.

1. No inferred holds

The agent must never infer that a medicine should be held, continued, bridged, substituted, or restarted. It may reproduce only a complete current patient-specific plan authorized by the responsible clinician/team.

2. Required peri-procedure dataset

Domain	Required information
Procedure	Name, site, date/time, bleeding/anaesthetic context, performing service, urgency
Medication	Exact name/formulation, strength, route, schedule, indication, last known dose
Patient factors	Renal/hepatic status, weight where relevant, bleeding/thrombotic history, allergies, recent results
Authorization	Responsible prescriber/procedure team, signed plan ID/version/time
Interruption	Last dose, hold start, doses omitted, bridging instruction if explicitly authorized
Restart	Condition/time/date, responsible authorizer, prerequisite haemostasis/result/assessment
Safety net	Bleeding/clot/symptom warnings and verified contact routes

3. High-risk categories

Situation	Mandatory review
Anticoagulant or multiple antithrombotics	Prescriber/anticoagulation plus procedure team; indication and renal relevance verified
Recent coronary intervention or time-limited antiplatelet plan	Cardiology review before interruption
Rate/rhythm or antiarrhythmic medicine	Procedure/anaesthetic and specialist review as applicable
Diuretic or potassium/renal-relevant therapy	Procedure/acute-illness plan with current observations/results as required
Implanted cardiac device procedure	Device/electrophysiology pathway
Unknown procedure plan or conflicting advice	CONFLICT_REQUIRES_REVIEW; no self-directed change

4. Temporary hold order

Element	Requirement
Medicine	Exact product/formulation and current schedule
Reason	Procedure, adverse effect, result, acute illness, or other documented reason
Start	Exact date/time or defined trigger
Scope	Which doses are omitted; no vague 'stop for now'
Monitoring	Tests/observations and responsible reviewer
Review/restart	Date, trigger, authorizer and prerequisites
Supply/list update	Status becomes HELD, not silently STOPPED; all lists reconciled
Communication	Patient/carer and relevant teams receive verified instructions

5. Conflicting instructions

- Preserve both sources, dates, authors, and exact disagreement.
- Do not assume the newest message is authoritative if status/authorization is unclear.
- Warm-transfer to the named peri-procedure owner or duty clinician according to urgency.
- Until clarified, state that no medication change is authorized by the system.
- For severe symptoms or major bleeding, follow the emergency policy rather than waiting for routine clarification.

6. Final-answer requirements

State procedure/date; current medication order; authorized last-dose/hold/restart instructions; warnings; required tests; responsible teams; appointment/contact plan; conflicts/missing data; authorization identifier; and citations. If any critical element is missing, withhold exact hold/restart advice.

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